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HEALING AND REPAIR IN DEMOCRATIC REPUBLIC OF CONGO

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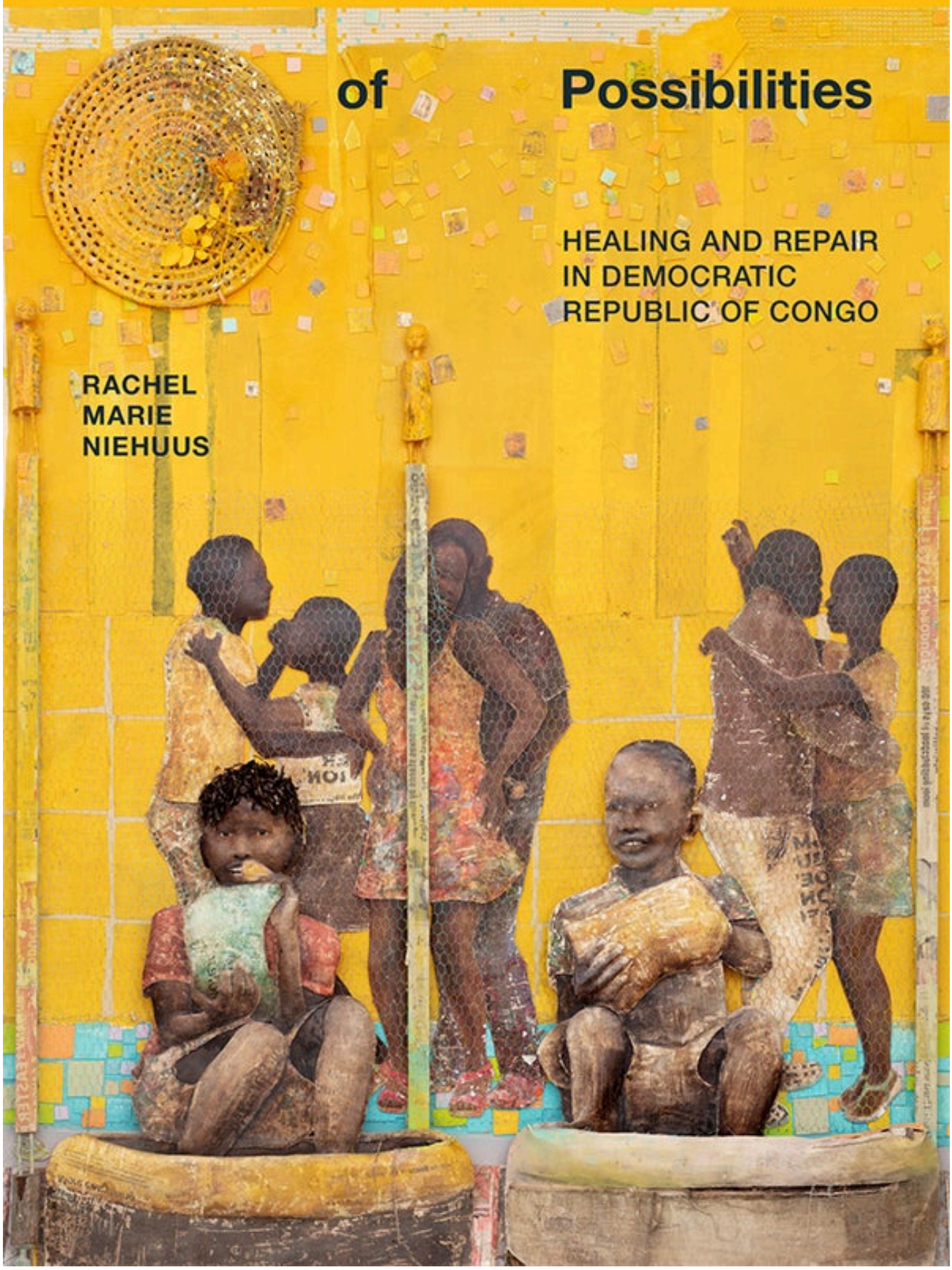

3

The Body, the Flesh, and the Hospital

How to care for the injured body,
the kind of body that can't hold
the content it is living?
And where is the safest place when that place
must be someplace other than in the body?
—Claudia Rankine, *Citizen*

In the absence of kin, family, gender, belonging, language, personhood, property, and official records, among other factors, what remains is the flesh, the living, speaking, thinking, feeling, and imagining flesh: the ether that holds together the world of Man while at the same time forming the conditions of possibility for this world's demise.
—Alexander Weheliye, *Habeas Viscus*

"Mwili, njoo yote," Solange said. The body, it is everything. I had just gotten home from work and taken a look at a red and inflamed lymph node that had appeared several days earlier under the chin of Solange's youngest child.

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FIGURE 3.1. The Kishabe Hospital, 2013. To the left in the photo is the pharmacy cabinet of the maternity ward; to the right is the unit's hand-washing station. The medication was often purchased through partnerships with various international humanitarian organizations. Because the water was carried in by hand, it was a precious commodity. Photograph by author.

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I had very minimal clinical training at the time, having only finished half of medical school; but Solange was intent on managing the abscess with compresses, and so I checked it every day to make sure it wasn't getting worse. Overall, Jo Jo was doing well, and as Solange and I sat watching him play with the other children, Solange expressed relief at having avoided the $5 consultation fee at the hospital. "Bandits might take your clothes and your crops. But, God willing, you can buy more clothes and you can find food. The body, though. The body is everything." *Mwili, njoo yote*.

This chapter investigates the role of the body in healing in eastern Congo. As Solange states, where material possessions and food security are so fleeting, the body represents that which one always has if (and when) everything else disappears. It is also the last, most intimate frontier of vulnerability. Where so few have access to safe food, water, or preventative care, sickness and injury are common, even expected; this even though healthy bodies capable of manual labor are necessary for survival. In eastern Congo, where healthy life expectancy has been estimated to be forty-four years, the body serves as undeniable proof of the poor standard of living that people share.[1] Both reflecting shared experience and demanding witness, the body opens up conversations about the possibility of a present otherwise, in which people and things can be counted on, in which life is less fragile.[2]

One note on terminology: in this chapter, I use "the body" to refer to the individual physical body; however, as the field of medical anthropology established long ago, many other bodies exist—the social body, the body politic, the body and the flesh.[3] Whereas, in Western epistemologies, these bodies—and the healing practices they require—are sometimes separable, in the Congolese context, the physical body has little meaning outside of its social context.[4] Thus, in a collection devoted to establishing the social basis of health and healing on the continent, historian/anthropologists Steven Feierman and John Janzen write, "Healing ideas and practices are not a separate domain. They are an integral part of politics, kinship relations, religion, trade, farming and sexual life."[5] Certainly, the specific ways in which some bodies fall ill in eastern Congo, and the forms of repair and healing that their affliction makes possible, tells more about the social condition of living in war than it does about the success (or lack thereof) of biomedicine.[6] As a physician, I have often found it difficult to observe physical pain, loss, and death in Congo and know that, elsewhere, other outcomes are possible. And, through my ethnographic work, I have learned that the act of inhabiting affliction is what allows the physical body to function as a register for repair, a site through which greater social healing might be demanded and presents

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otherwise imagined. In this chapter, I seek to portray both the devastation and the possibility that accompany illness and the demands for bodily healing.

The Hospital: A Site of Healing or of Harm?

The first Belgian excursions in Congo were oriented toward morality—stopping the Arab slave trade—territorial control, and profit. With the field of public health still nascent in the metropole, the imperialists devoted little attention to improving the health of the Congolese.[7] Instead, hands were severed in forced labor campaigns, and the porters who died of hunger, exhaustion, and infectious disease were simply replaced.[8]

By the early 1900s, photographs of imperial wounding had begun circulating in Europe. As news of the atrocities committed in Congo spread, another iteration of Western intervention was proposed: perhaps the natives could be civilized; perhaps they could be saved from their Godless ways.[9] In 1908, King Leopold's personal grip on the country loosened as the Belgian parliament assumed control of the country. Shortly thereafter, in the words of historian Nancy Rose Hunt, "medicalization became a form of colony building."[10]

By 1910, mining companies had begun rewarding employees who had more children with food support, bonuses, and tax breaks. These pronatalist measures accompanied vaccination efforts and institutionalization of the traveling mission doctor and marked the beginning of hygienic rule. Over the next decades, the footprint of the colonial state grew, and Belgian authorities built an extensive network of health centers and maternities. As they sought to transform health from a traditional practice to a modern one, they used coercive and forceful practices to wrest authority from Congolese healers and move healing into the newly constructed centers.[11] Throughout this process, birth work came to occupy a unique place in the Belgian colonial state. Given the population decline that followed the brutal imperial period, state health officials, religious authorities, and company leaders advanced a pronatalist agenda. In an effort to shorten birth intervals and achieve a higher birth rate, traditional forms of birth spacing were outlawed and monogamy, breastfeeding schedules, and early weaning were institutionalized. By 1958, the Belgian Congo boasted the most extensive medical infrastructure in post–World War II Africa. By medicalizing birth, the Belgians successfully reversed population decline, thereby producing a large and robust labor force. They also expanded the reach of the church and

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the state, with maternities serving as an important opportunity for evangelization and for bureaucratic inscription.[12]

And so, the Belgian colonial state, together with its Congolese middle figures, built world-class maternities and laboratories; but the intent had never been to create an autonomous health system. When Congo obtained independence in 1960, not a single Congolese physician had been trained.[13] In the political upheaval that followed independence, many Belgian nationals—including much of the health care force—left the country. For nearly a decade, health care in Congo was piecemeal, provided by a combination of Belgian missionaries, Congolese nurses, and other international humanitarians.[14] In the early 1970s, the nation's health finally drew the attention of then-president Mobutu. Taking seriously the idea that nationalized primary care would strengthen the people, and thus the country, Mobutu worked to develop a nationalized, decentralized health care system.[15] For a short time, the national government expanded to provide clean water, sanitation, treatment of most common ailments, maternal and infant health, family planning, and essential medicines to all Congolese free of charge; however, the state collapse that began in the early 1980s spared nothing. As he attempted to cut expenditures from the state's quickly emptying coffers, Mobutu first minimized public investment in the national health care system.[16] Several years later, he tightened the budget further, introducing user fees to help keep the doors open. But his hold on power was quickly unraveling, and local health care administrators soon began seizing these user fees for their own enrichment.[17] Within a decade, the robust nationalized health care system had been transformed into the weak and often predatory market-based system that exists today.[18]

The Kishabe Hospital, eastern Congo's first hospital, was built in 1963. For the first twenty years of its operation, it was overseen by Belgian physicians and supported by a combination of Belgian donations and state money.[19] With funding from a Belgian nonprofit, a nursing school was later established nearby, and the hospital became a regional training site for both nurses and physicians. During this time, people traveled hundreds of miles to Kishabe to receive what was widely considered to be the best health care in eastern Congo. In addition to providing health care, the hospital also offered formal employment, the first of its kind, to residents of Kishabe. Men and women who had grown up picking coffee on Belgian plantations were educated and employed as nurses in the hospital; others took up custodial duties; and still others operated small stores in the market that arose to

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support the hospital's needs. Over time, the hospital became the focal point in the small farming town.

In 1976, a Belgian cooperative named CENAB began supporting the operation of the hospital as part of a project aimed at rehabilitating health care in the eastern Congo.[20] Still, by the early 1980s, the nine-building complex had begun to show evidence of "the Zairean sickness," *le mal Zaïrois*, that plagued the rest of the country.[21] When state collapse became more imminent in the 1990s, the state's role in health care became limited to "a legal framework for non-state actors," and international NGOs entered en masse.[22] At various points in the 1990s and again in 2007 and 2008, during periods of brutal fighting in and around Kishabe, Médecins sans Frontières (MSF) labeled Kishabe an emergency zone and took over full operational support of the hospital. Before and since then, operations at the hospital have been supported by a bricolage of different national and international entities: Ministry of Health, CENAB, UNICEF, Save the Children, World Food Program, and HEAL Africa. Despite receiving a relatively high level of support, the Kishabe hospital often lacked the personnel and supplies it needed to help those who passed through its doors. As a result, the hospital harmed as much as it healed.

The *ô* in *Hôpital* has fallen off the sign out front of the building; the paint is peeling, the cement cracking. Many windows are broken, and the roof leaks in places. Water is all over the floor, but never from the spigots. Some of the bed frames date to the hospital's construction; others, broken, litter the courtyard. The lab has a light switch but no light bulb. In order to take an X-ray, families first have to buy gasoline to start the generator. Episiotomies are done with a kerosene lamp. Babies are born on the hospital's single metal obstetric table, on which sits a thin, torn piece of foam covered by a piece of vinyl. In the dry season, when the air is thick, the smell of pus clings to this vinyl, even after a dousing with bleach. In spite of the infusion of hundreds of thousands of dollars of humanitarian and development aid, the hospital appears weary with decline.

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In the Kishabe hospital, as in any biomedical space, cleanliness was everywhere attempted: surgical (fabric) towels and drapes were carefully washed by hand, dried in the sun, and then autoclaved; disinfectant was liberally applied to the floors in the operating room and the wards; patients were meticulously washed and shaven before surgeries; surgeons scrubbed their hands until the skin cracked and then doused them with alcohol. Despite all this effort, however, dirt entered the hospital. Patients walked in barefoot,

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with mud caked on their feet. Fine granules of sand clung to the clothes of visitors who had traveled many miles to see sick family members. The afternoon breeze brought the black earth from the vertical farmland surrounding the hospital through the open windows. And in a context in which supplies, water, electricity, and time were always in short supply, dirt worked its way into bodies.

Upon entering the maternity ward, I heard the restrained cries of a woman on the obstetric table. Instead of the deep, sustained sounds of childbirth, the cries that emanated through the tarp that separated the nursing station from the obstetric table were staccato and irregular. Lydie, a nurse-midwife, stood on a stepstool, neck craned over Pascaline, a woman who had recently delivered via cesarean, examining Pascaline's belly at close range. What should have been a neat line of stitches appeared red, swollen, and disfigured. Infection had set in. Lydie stood back up and began squeezing both sides of the incision toward each other, as if trying to pop an abdomen-sized pimple. The depth of the infection required significant force, and Lydie's jaw was set for the effort. Pascaline writhed in pain and then let out a visceral howl. Lydie sternly chastised her for moving; Pascaline inhaled sharply and lay still. As Lydie pressed even more of her body weight into the wound, Pascaline held her breath. Finally, a yellow line of pus erupted from the site. Lydie relaxed momentarily. To Pascaline, she said, "See, I told you it was dirty [*mchafu*]. I have to keep going now; we have to get all of this out of there. It will never heal if it stays dirty."

For several agonizing minutes, Lydie moved along the full length of the eight-inch incision, pressing deeply around the sutures to break up any pockets of infection. Pascaline breathed rapidly and unsteadily. Several times, she moaned, then hummed in pain. With her arm covering her face, she turned her head left, right, then left again as if refusing both the procedure and the pain. Several lines of blood-tinged pus ran down her belly and met the table beneath her. When the wound stopped producing discharge, Lydie relaxed the pressure. With great relief, Pascaline exhaled.

A combination of the infection and the squeezing had torn the skin around several of the sutures. Where once a line of black stitches had held two pieces of flesh together, there was now a half dollar-sized hole. Lydie used disinfectant-soaked gauze to clean the hole before beginning to pack it. As she pushed piece after piece of gauze into the dark hole, Pascaline let out a quiet cry. At some point, the wound stopped accepting gauze, and Lydie applied a clean bandage over the incision. Then, she gestured for Pascaline to get up. Pascaline slowly curled to a stand. She took a breath, then, grimacing

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in pain, started to get dressed. As Pascaline walked out the door, Lydie said sternly, "Come back tomorrow, and every day for a while." With the wound infected and now open, it would have to be cleaned and packed on a daily basis until the pus gave way to new, pink flesh.

For the next thirty-seven days, Pascaline joined the long line of women who returned daily for the cleaning of their infected incisions. Sometimes she was turned away for lack of time or clean bandages. However, on most days, she endured a painful, prolonged dressing change—the pieces of gauze had to be fished out piece by piece—before returning to the common room, where she waited for the infection to pass so she and her baby could return home. As the days became weeks and the weeks, a month, Pascaline's figure returned and her baby grew a head of hair. Still, her incision remained open and oozing. Finally, to expedite her recovery, a physician decided to sew the open area of her incision back up.

The secondary sutures were placed on the obstetric table in the maternity ward. Dr. Alex began the procedure by injecting the area to be sewn with local anesthetic. Pascaline flinched but remained silent. Dr. Alex waited a few moments for the anesthetic to take effect, then began sewing. As he placed the first stitch, Pascaline wailed and grabbed for his gloved hands. Wary that Pascaline might introduce further infection into the wound, Dr. Alex called for nurses to restrain Pascaline. "I put lidocaine in," he said, confidently. "It will work soon." Then, with three people holding Pascaline down, he continued to sew up the hole in Pascaline's abdomen. At first, Pascaline struggled; then she wailed; then she fought again. One nurse barked orders at her: "Stop moving! Be patient!" Another offered more gently, "Close your heart. Stop crying and be strong." Dr. Alex sewed resolutely, stopping only to slap Pascaline's leg and repeat the nurses' orders. For several minutes, the torture scene continued: Pascaline, restrained by three nurses, wept; Dr. Alex berated her for being emotional and continued to stitch. Finally, he tied the last knot. As he put his instruments away, Pascaline continued to cry. Dr. Alex shook his head and looked at Pascaline in disgust. On his way out of the room, he chided the nurses: "I'm not coming back in here again, if this is how your patients behave." When the door closed behind him, the nurses turned to scold Pascaline. "We should make you pay a fine," they said. "A woman is she who silently toils for her family's benefit [*mwanamke ni kufunga kamba kichwani*]. You must learn to close your heart. Suffering is a valuable lesson [*mateso njoo masomo*]." Pascaline got dressed and, with tears still streaming down her face, walked out of the room.

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In the Kishabe maternity ward, pain was considered an expected part of any intervention. As a result, nurses never asked about it, and patients seldom mentioned it. Instead, whether it was labor pain or the pain of wound care, it was most often endured quietly and individually. On rare occasions, women who had just returned from the operating room were given Tylenol to take the edge off; stronger pain medications were reserved for exceptional cases—traumatic injury, orthopedic manipulation. Sometimes, as Pascaline's story demonstrates, no pain medication was offered at all. But this is not to say that pain wasn't acknowledged. In fact, quite the opposite. After wound care or during the later stages of labor, nurses steadied patients as they got down from the table, helped them get dressed, and walked them back to their beds. When the pain was too great for a woman to move or to speak, nurses waited silently, patiently until it passed. When younger women cried out in pain, as Pascaline had, the nurses first encouraged them: "Be patient, stay strong, close your heart [*funga roho*]," they said. If women continued to cry out, nurses (and doctors) sometimes slapped their legs or their arms. Though occasionally done out of anger, this was most often done in an effort to discipline, to teach. As one nurse said to Pascaline, "Suffering is a valuable lesson." In response to my questions about hitting during labor, another nurse said, "If a woman learns to bear suffering quietly now, when war comes and she is in labor, she will be able to birth a child on the side of the road and get up and run again. We are teaching them how to survive."[23]

Since the colonial era, the hospital in Congo has intervened on more than the physical body. Of colonial medicine in northeast Congo, Nancy Rose Hunt writes, "Lowering maternal mortality was never the express purpose of colonial midwifery at Yakusu. Evangelization, 'getting the girls,' and producing a 'trained staff of female nurses' were the missionaries' goals."[24] Where many (still) lacked a formal education, the hospital in present-day Kishabe also performed an educational role. When women presented without birth supplies—a plastic sheet on which to birth, a bucket that would serve as a chamber pot, a piece of fabric to swaddle the newborn—nurses instructed them on "proper" birth supplies and "proper" birth etiquette. When women left the hospital after delivery, the nurses reminded them about "proper" ways to wash (away from the family so no one would be bothered by the smell) and "proper" ways to avoid falling pregnant too soon (abstinence). Much of this talk felt disciplinary, occurring as it did down a power gradient, characterized by a sense of right and wrong, and sometimes accompanied by a slap or a harsh word. Because childbirth made women

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the most frequent users of health care in Congo, it was always highly gendered. And, if one looked to nurses' physical efforts—Lydie's determined squeezing of Pascaline's wound, the pushing and pulling midwives engaged in to deliver a stuck baby—a profound sense of duty, a deep dedication to one's brethren came into view.

Over the thirty-seven days that Pascaline underwent wound care, the whole team of nurses took part in the very fleshy work of pressing down, digging in, and bandaging the festering wound.[25] As they cared for Pascaline's wound, these nurses also taught her about pain. For, both nurses and patients claimed, if a woman could learn to bear physical pain in the hospital, she would more gracefully be able to carry the other (physical and emotional) burdens of life. Where daily life often involved hunger, heavy work, and violence, fostering fortitude—training women to endure severe pain silently—equipped them for life in war. And so, as they chided and pushed and slapped, thus blurring the line between harming and healing, the nurses in Kishabe did what they could to deliver mothers and babies to safety while also ensuring that their patients had the tools they needed to survive in the longer term.[26]

"Ninakufa": Dying and Transformation

Since its construction, the hospital has been composed of four wards: internal medicine, surgery, pediatrics (which also housed the malnutrition ward), and maternity. Because the maternity service was the busiest in the Kishabe hospital, I spent the majority of my time there. Although its census often reached seventy-five—a handful of gynecological cases, thirty or so recently delivered women, and about the same number of women with high-risk pregnancies who had come down from rural areas in their final weeks to give birth—only two nurse-midwives were on staff at any one time.[27] As a result, the ward was often bustling. Patients waited to be admitted, discharged, examined, and served their medicine; beds filled up too quickly and emptied too slowly; blank charts piled up. On especially surcharged days, the nurse-midwives half-joked that their assignment to the maternity service was punishment.

In addition to being busy, the service was unpredictable. The only sure thing about days in maternity was sign-off from the night crew at 7:30 a.m. After that, the day's activities depended on whoever walked in the door. Some days were occupied with routine tasks such as medical rounds,

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dressing changes, paperwork, and prenatal visits for the high-risk pregnancies; other days included deliveries, miscarriages, hysterectomies, and obstetric emergencies. Since Kishabe's was a referral hospital, most of its patients had passed through one of the smaller health centers in the hills first. Often, they had complications with deliveries and needed a higher level of care; however, with no state-run transport system, many had to walk or arrange other transportation between the health center and the hospital. By the time they arrived in Kishabe, many were gravely ill, and about half required emergency surgery.

The common room in the maternity ward was one of the most active places in the hospital: patients' family members, who did most of the care work and nursing, laughed together as they doted on their charges; mothers murmured to their newborns as they nursed; and babies became ruddy-faced in their protests against being bathed, changed, or dressed.[28] At the far end of the long, rectangular block, women with long faces and feverish babies knocked impatiently on the delivery room door. From behind the door, a nurse-midwife barked that she couldn't see the sick babies yet—and so the line of women holding their newborns grew. Across the aisle, women who walked with the hunch of a new incision did their own knocking. As the din grew louder, a nursing student came out to disperse the waiting women. "The bandages haven't been sterilized yet," he shouted. "No bandage changes until after noon." But for every woman who went back to her bed, another one stood up.

In the next room, a delivery was always in progress. A slap, a stern voice, and then the sounds of a delivery penetrated the tarp that surrounded the obstetric table. From within the womb to a world of tarps, plastic sheets, and metal instruments. But she wouldn't stay there for long—now that the baby had proven herself strong, she would be wrapped in onion layers of soft blankets and danced around the room in joyful celebration. At least this one, at least on this day.

On Monday of my second week in the Kishabe hospital, I walked into the maternity ward as shift change was starting. The off-going midwife began: there had been a "a catastrophic case." Her first baby had been a term stillbirth. (Where few women have access to prenatal care, this was a too-frequent occurrence.) Then, during the seventh month of this second pregnancy, she started hemorrhaging suddenly. Her family brought her to Kishabe, and the doctor tried to stop the bleeding. The fetus lost a heartbeat, and the woman continued to bleed. Where there was little capacity for blood transfusion,

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the on-call doctor performed a hysterectomy to stop the bleeding and save the mother's life. She is alive for now, the off-going midwife reported. "But watch her closely."

After several clucks from the incoming team, one of the incoming midwives recounted the story of another tragic case from the weekend. A woman had given birth at the health center in a town five miles away. After delivery, she began hemorrhaging. They gave oxytocin and did uterine massage, per the protocol for postpartum hemorrhage. When she kept bleeding despite these interventions, they began the process of transferring her to a higher level of care. According to the nurse telling the story, the nurses at the transferring health center had taken several hours to prepare her paperwork. On their end, the family had taken a few hours to come up with the money necessary to pay the first health center and rent the bus that they would use for the transfer. More than twelve hours after giving birth, the woman finally arrived at the emergency room in Kishabe. The Kishabe staff had worked quickly to mobilize an operating team and an operating room. But the woman's blood pressure was already too low, and the single IV she had in her arm too small. She had died shortly after rolling into the operating suite.

For a minute, the room was quiet as all present digested the two stories. Eventually, the nurse-midwives continued sign-out: there were three women laboring, dressing changes to do, and two other women on the ward who had undergone cesarean sections overnight and still had low blood pressure. As the incoming team began preparing for the day, the off-going team finished their charting and left. The bustle began, and still a sense of the fragility hung in the room.

After report, I rounded with the team and attended several (live) births. Every time I walked out of the nurses station into the common maternity room, I was cognizant of the fact that the woman from overnight, who had lost her baby and her uterus at once, was the only woman in the room without a child. At some point, after her blood pressure had come up and she was able to sit, I went over to extend my sympathies. "I'm so sorry," I said. She looked at me with tear-brimmed eyes and took a moment to compose herself. Then she bit her lip and murmured, "I am healed [*nimepona*]." It was the response I expected—the one that traditionally followed sympathies in eastern Congo—but what could healing possibly mean in this context after such a tragedy?

One week after the first that I recorded, there was another neonatal death in Kishabe. Four days later, I recorded another. In the same week, I watched three neonatal resuscitations.

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The next two neonatal deaths took a month to tally.

There was a baby who got ampicillin. The mom had said, "My baby is warm and she is not eating." The neonate was two days old. "Bring me a spoon and some water," the midwives said to the mom. And then they picked out a tablet of ampicillin from the bottle, cut it into fourths, and crushed it on the dirty table. They used a paper to lift the powder from the table to the spoon and then put some water on the spoon and held the mixture to the lips of the lethargic, febrile child. The next day, the baby was a bundle on the floor, waiting for her parents to pay her hospital bill so that she could go to her own funeral.

At some point, I began counting bundles on the floor. Many days there are none. One day there were three.[29]

It was shift change. The loud humming of the oxygen condenser threatened to drown out the handoff between the night nurse and the women coming on duty. There had been a cesarean, a baby boy who had come out screaming. But he had deteriorated in the past few hours and now needed oxygen.

Justine's report had been brief, to the point, and she was out the door before we said goodbye. I got up to visit a woman who had lost her child the day before. As I walked toward the maternity ward, I passed by the crib in which the struggling newborn was lying. The baby's brightly colored swaddle had fallen over his face, and I brushed it away, eager to see a new little ruddy face. Instead, as my hand pushed back the *kikwembe*, I saw blue lips. The oxygen condenser had shut off at some point without anyone noticing. "Anne-Marie, Anne-Marie!" I screamed. "ANNE-MARIE!"

As I clawed frantically through the layers of blankets, Anne-Marie came running from the other side of the room. Somehow her hands reached past mine. She grabbed the baby by the ankles and pulled him out of the crib and upside down in one motion. As she slapped his back, I rummaged on a shelf below for an oxygen mask. And then the baby was back in the crib and Anne-Marie had started CPR. While her hands worked, her eyes searched the shelves for a vial. When she found what she was looking for, she stepped away, grabbed a syringe, and drew from a half-full vial. While Anne-Marie was away, I squeezed the oxygen bag in small, quick bursts, then took the child in my arms for another round of CPR. As I alternated back to the baby's head, Anne-Marie skillfully injected epinephrine into his umbilical vein. Then she went back to chest compressions. As we worked, Anne-Marie looked nervously at the clock and pleaded urgently, "Come on, come on, little one."

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Fifteen minutes elapsed. Anne-Marie's hands stopped. We both knew that if the baby didn't make it now, he wouldn't. We gazed down expectantly at the baby in the crib. Slowly, the baby's face contorted, and he let out a weak cry. Anne-Marie and I looked at each other with tears of relief. This one would make it. Several hours later, his screams would become loud and rigorous, and, having become a disruption in the delivery room, he would be given back to his mother to hold. Meanwhile, the three other bundles on the floor would disappear and would never be spoken of again.

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At some point, I discovered that the hospital did not keep records of infant deaths, those bundles on the floor. Stillbirths were captured—in the birth register, there was a column for the status of the infant, and when the fetus was already dead, this column contained an M for *mort-né*. But the birth register was usually filled out immediately after delivery, and there was not a separate register that listed infants as patients. This combination of facts meant that babies who were born alive but died shortly afterward—the majority of the bundles on the floor—were never tallied. I tried to think of how many bundles I had seen, how many cold bodies I had personally wrapped. When I couldn't come up with a number, I started keeping track. Between February and June 2013, my field notes are punctuated by numbers: 0 bundles, 0 bundles, 1 bundle. On the days that I did not go to the hospital, I wondered how many bundles I missed. As I thought about this tide of bundles that no attempt at counting would stop, I wrote:

> How many times will I watch them reach for bicarbonate? Today, there was no gas for the generator and the power had gone off, or perhaps it had just been too long since someone had checked on the fragile new being. At some point, the baby had stopped breathing, and now he was blue and cold. The covers are pulled back from his face and chest, two fingers pump his chest, too fast, but maybe this is what it meant to jump start. It's always an emergency when then they reach for the bicarbonate. But how many emergencies can a body take?

In my quest to tally the deaths, I descended to medical records, an office at the bottom of the hill. I made Excel sheets from the hospital's monthly reports that were submitted to the state. From observing how the midwives filled in registers, I knew the numbers would only be partial. Still, it seemed important to quantify the loss. For the period April to June 2013, I recorded: Births: 60, 71, 52; Cesareans: 44, 32, 32; Stillbirths coming from health center: 5, 7, 3; Stillbirths during labor: 0, 0, 0; Neonatal death: 1, 2, 0;

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Maternal death: 1, 0, 0. Looking back over the last two years, this three-month period—in which there were eighteen neonatal deaths, one maternal death, and three hundred deliveries—seemed typical. I put the numbers into a calculator. Officially, nearly 6 percent of all births in Kishabe ended in death of the baby. And this was not counting the six other bundles I had tallied during this time.[30]

When I discussed my math with the midwives, they were adamant that they were not to blame for this number, which, if generalized across the country, would mean that Congolese neonates were ten times more likely to die than infants born in the United States. We work so hard to save them, the nurses maintained. But the running and hunger, the inability of the Congolese state to care for and protect its citizens, put limits on what they could do in the hospital, they insisted. Besides, this was not a new phenomenon, or specific to Kishabe. "In Congo, we have died like animals for years and no one has cared," they argued.

It is true that elevated infant (and maternal) mortality rates have a longer history in Congo. In the precolonial period, before the Belgians built maternities, childbirth was often likened to war: "childbirth is dying," the saying went; "giving birth takes women to the mouth of the grave."[31] Under imperial rule, dying became more generalized. Some were worked to death; others saw violent ends. At some point, the loss of able-bodied laborers combined with high rates of infertility to engender a Belgian panic over population decline. Would there be enough hands to pick rubber, to sift for gold? Fifty years later, the pronatalism instituted during this time lingers. Despite evidence that increased parity increases both neonatal and maternal mortality, the hospital's family planning offerings were characterized by supply chain ruptures; and even when Depo-Provera was available, midwives would not inject it without a husband's signature. And so, where many women presented with their sixth, seventh, eighth pregnancy, often after long labors elsewhere, where prenatal care was a rarity and life-saving blood products were more than an hour away, despite midwives' efforts otherwise, giving birth still took women to the mouth of the grave, and in Kishabe, laboring women still said, "Ninakufa": I am dying.

But the medical aspects of maternal and infant mortality are only part of the story. Anthropologist Filip De Boeck writes, "Without children, there are no gifts. Without gifts, there are no kin, no allies or social body."[32] Everywhere, children are figures of social reproduction. However, where so many households and communities have been fractured by war, children's ability to build new kinship, new connection, affords them a unique position in the

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social fabric. In a regime of nothingness, children offer a potential exit—toward relationality, recognition, a future that can be counted on. When I asked the midwives what they liked about their jobs, they stated, "Our children are our tomorrow"; "Our children allow us to dream [*kulota*]."[33] And yet, the oxygen rarely worked, the bandages were rarely clean, and bundles continued to accumulate on the floor. How to reconcile these two facts?

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In "Variations on the Beautiful in the Congolese World of Sounds," Achille Mbembe traces the well-being of the Congolese people through different eras of Congolese music popular at different times.[34] First, he examines the choral music popularized during Belgian rule. He writes, "One of the most important contributions to Congolese music made by Christianity proved to be aesthetic indoctrination—an imposition of view as to what constitutes beauty, rules concerning correct harmonic progression, voice leading, types of vibrato and timbral characteristics."[35] Surely, Mbembe posits, other music existed during colonial rule—and many other subjects, too—but what was recorded were the voices that fell in line, adhered to the standards set in the metropole.

After choral music came the rumba, that buoyant genre popularized in the celebrations that marked Congolese independence. Mbembe asserts, "In many respects, [postindependence] music epitomized joy, festivity and happiness, elegance and serenity. It enabled the Congolese to sing what cannot be spoken about in any other kind of speech."[36] But soon after independence came the crushing weight of rock-bottom copper prices, crippling structural adjustment policies, and the heavy hand of a nervous dictator. By the 1990s, joblessness, hunger, and state violence were widespread. It was in this context, Mbembe posits, that contemporary Congolese music emerged. He writes: "Screaming, howling, throughout the last quarter of the 20th century and into the new millennium, part noise-sound, part musical scream, Congolese music has endeavored to account for the terror, the cruelty and the dark abyss—for the ugly and the abject—that is its country."[37] Where Congolese music has always been characterized by jubilation, ecstasy, the integration of the scream into music has allowed for a certain truth telling about the terrible, violent realities in which most people live. As it has freed dancers from the usual deception and ostentation that characterized the Congolese musical world, Mbembe argues, the scream has opened public space for people to inhabit the pain and loss that characterize their lives and bear witness to others doing the same.

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Mbembe writes about Congolese music; but I have heard the scream that he describes, which reflects not only physical pain but also the "the terror, the cruelty and the dark abyss—for the ugly and the abject—that is its country," echoing through the walls of the Kishabe hospital. There was pride in grieving silently, and so many women did; but some mothers whose babies had become bundles on the floor screamed in the middle of the night, or in the showers, or when they got dressed to return home. And after she told me, "I am healed," once I had walked away, the woman who lost her child and her uterus on the same day uttered a gutteral "God, oh God" before falling silent again. In the realm of music, Mbembe suggests that, where crushing poverty and violence continued to threaten erasure, the socialization of suffering—the witness of the scream—is itself a form of healing.[38] Where life outside the hospital is so very hard—the hunger, the violence, the running, the fear—and the hospital cannot stem the tide of injury and death, I wonder if, more than saving individual lives, the central function of the Kishabe hospital might be the recognition of suffering and rendering it social? If one were to look only at the numbers, the Excel sheet of infant deaths that I made, it would be easy to chastise the nurse-midwives, to wonder why women returned—and many funding organizations did this. But if one listened to the nurse-midwives—"In Congo, we have died like animals for years and no one has cared," they said—if one sat with their frustration, recorded their exhaustion, witnessed their efforts, a different conclusion might be reached. It was not that the midwives were callous, or that the mothers they delivered did not notice the losses. Surely, the deaths of tomorrow were acutely felt—and shared—by all in the Kishabe maternity ward. But where oxygen was always scarce and clean bandages numbered, perhaps the witnessing of loss, of grief, of the screams that could not be hidden from all who slept in that large common room was itself a healing intervention.

Mtunzo and the Hospital's Promise of Care

At the foot of the hospital's steps, a handful of people operated small, wooden boutiques that provided the goods necessary for a prolonged stay: batteries for radios, quarter-sized bags of powdered soap, Fanta, and fried dough. A dozen women gathered in the open area in front of the boutiques to sell tomatoes, onions, and ten-cent denominations of powdered milk, sugar, and dried fish to patients well enough to walk to the market from

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their beds. At any one time, several young men waited with their moto-taxis at the foot of the steps for the next discharged patient who would become their next client; anyone without work gathered under the shade of the hospital's eucalyptus trees to relay news and exchange gossip.

The town didn't stop at the hospital entrance; rather, the hospital itself was deeply imbricated in the town's social and economic activity. At all hours of the day, a seemingly constant flow of people circulated through the wards: female friends and family delivered food and soap for the sick; fathers and husbands arrived to pay the bills of their charges; and counselors and pastors visited the sick and accompanied those who had been discharged home. In addition, women sold fish from baskets they carried on their head through the wards; children peddled kerosene from old water bottles on the walkways that connected the wards; and merchants hawked clothes, wax fabric, and plastic goods on the hospital steps. When night came and stillness fell over the town, the activity continued in the hospital: well after dusk, family and friends shared food with those who were hospitalized; after the food had been put away, mothers, wives, and daughters washed the sick and tucked them in before rolling out straw mats on the concrete between the hospital beds and taking their own reprieve.

Regardless of whether they entered as a patient or a visitor, a vendor or an employee, everyone in Kishabe spent time in the hospital. Above, I have explored the role of the hospital in relation to its patients, the way that it taught about physical pain and witnessed loss. But the hospital exerted a magnetic pull on the entire surrounding community, not just on the afflicted. And so, as I work to untangle the knot of the hospital that drew people into its gates despite the screams, the losses, in this section, I examine a more metaphysical kind of care, of healing. Though less corporeal, less intimate, it is a kind of care that continues to be brokered by the body and its vulnerability in war.

At less than five feet tall, Antoine was a slight man. The faded, threadbare blue coveralls that he wore swallowed his body, and his too-large white rubber boots reached almost to his knees, making him appear even smaller than he was. Born in Kishabe, he had grown up farming. For years after his father died, Antoine had continued cultivating the large plantation of banana trees that his father had overseen. In 2007, after he had married and become a father, pests came through the area and killed the banana trees. Finding himself unemployed and with many mouths to feed, Antoine began looking for work. It was 2006; war was everywhere, and MSF was running the hospital.

The high salaries that MSF offered lured him to the hospital in the first place, when, as he recounted, "the hospital was still clean." The work was

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physically demanding—he prepared the operating room prior to surgery and attended to the equipment and surgical supplies during the operation. And due to the frequency of surgical emergencies—so many gunshot wounds and obstetric complications—Antoine often found himself working seven days a week. But the regular salary of $100 a month that MSF paid enabled Antoine to feed his family. And with time, he was able to buy a plot of land and begin the construction of his house. "Farming kept us full," he remembered, "but with the hospital work, I could finally build."

In the 1960s, employment in the hospital had been prestigious, one of the few opportunities for upward mobility available to rural Kishabe residents. Nurses owned land. Some even drove cars.[39] During Mobutu's rule, the hospital, like all public institutions, had been hollowed out, and salaries became intermittent. In many ways, MSF's tenure at the hospital turned back the hands of time, to a time of prosperity and construction. However, when conflict died down and MSF left (at least for some time), working conditions at the hospital again deteriorated. Antoine remained one of the few people in Kishabe who was formally employed. And yet, because the hospital paid so little, his family often only ate once a day, and his children did not go to school. "We have such a bad life," Antoine said. "At least before, we could live, we could survive. Now, we have nothing." Instead of delivering on Antoine's expectations for a better life, the hospital rendered him hungry, frustrated, and in debt.

Antoine and I met on the hospital stairs in my early days in Kishabe, where I had descended to take a breather from the maternity ward and he sought cell phone service. At first, we crossed paths sporadically. With time, however, our paired descents to the stairs became more intentional. Antoine had a knack for animated dramatizations of precarity: like a jester, he pointed out the holes in the clothes of people who walked into the hospital and then put his own threadbare clothes on display as comic relief. With wild gesticulations, he told of the bills that he had that he couldn't pay—and, as if he were the host of a game show, asked those waiting at the accountant's door what they had sold to pay the hospital bill of their family members. By making light of the poverty, illness, and loss people were bearing, he transformed the mood at the foot of the stairs.

But Antoine had a quieter side, too, when he was deeply concerned about his family, how they were all going to get through. On one such occasion, I asked him why he continued to work at the hospital if it didn't pay. Why not return to the land? At least then, you could feed your family, I suggested. "At least, through the hospital, my family gets *mtunzo*," Antoine replied,

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repeating a refrain I had heard from others like Antoine who struggled to feed their families with the salaries they made from the hospital. In Swahili, *mtunzo* refers to physical care, health care, and/or the cost of this care; it also describes a more metaphysical, social concept, taking care of the sick and the vulnerable more generally. Antoine did not clarify what kind of *mtunzo* the hospital provided, but I believe that this slippage is precisely the point. If Antoine or his family were to fall sick, the hospital would pay their bill. In this way, it provided health care. And, through his daily work in the hospital, his ethical mopping and sterilizing, Antoine imagined that one day, the hospital would deliver a better life for him and his family. He wasn't sure how this was going to happen—perhaps MSF would come back and provide him with the means with which to finish his house—but even if not MSF, another organization would intervene and reward him for his long suffering, for his years of uncompensated toil. One day, Antoine insisted, the care that he provided to the hospital and its patients would be recognized, his own suffering seen, and a dignified salary paid. In this way, Antoine imagined the hospital as the doorway to a more metaphysical form of care, of healing.

Black critical theorist Hortense Spillers writes about the division of Black flesh from the body that has taken place in antiblack regimes. She writes, "before the 'body,' there is the 'flesh,' that zero degree of social conceptualization that does not escape concealment under the brush of discourse or the reflexes of iconography."[40] In Spillers's writing, flesh is captive, seared, whipped, deprived; whereas the body is liberated, self-possessed, recognized by legal personhood. Centuries have passed since the emancipation of enslaved people in America; however, the racializing assemblages that initially cleaved Black flesh from the body continue to organize life across the globe. Moreover, capitalist modernity is built on this division.[41] And so, Antoine has a sense that another present is possible but lives a very fleshy everyday. Because of his proximity to the hospital, however, Antoine's story does not end at the flesh. Rather, where he has seen the hospital provide for others in the Kishabe community—a stairway to a life beyond the hospital's walls—Antoine dreams that, through the connections with humanitarian organizations and NGOs he makes in the hospital, he might finish his house, provide three meals a day for his family, clothe his children. Through his ethical work at the hospital, he might one day live as a social subject in a regime of the flesh. For Antoine and many others who pass through the hospital's gates, this possibility, that the hospital might enable a suturing back together of Black flesh to body, a repair of the centuries-old wound, motivates return.

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Too often, the Kishabe hospital failed to keep alive those who sought care at its doors. Decades of neglect, scarcity, and violence in Kishabe had taken their toll, and deprived of the human or material resources necessary to curb the precarity of life in war, the hospital could not stem the death and debility that passed through its doors. In 2007, members of an American NGO attempted to quantify the human cost of war in eastern Congo. They reported that between 1998 and 2007, war caused 5.4 million "excess deaths" in Congo.[42] Of these 5.4 million, the great majority were not direct results of conflict, but rather the consequence of nonviolent, treatable, and easily preventable illnesses such as pneumonia, malaria, diarrhea, malnutrition, and obstetric complications.[43] Despite the efforts of Antoine, Anne-Marie, Lydie, and others, these excess deaths continued in Kishabe long after Coghlan et al.'s report. And still, the Kishabe community rotated around the hospital. In this chapter, I have sought to reconcile these conflicting facts—that as bundles piled up on the floor, more people streamed through the hospital's gates.

In the epigraph that opens this chapter, Alexander Weheliye writes that "what remains is the flesh" in dense antiblackness, "the living, speaking, thinking, feeling, and imagining flesh: the ether that holds together the world of Man while at the same time forming the conditions of possibility for this world's demise." Today, the hospital in Kishabe continues to hold together the world of Man, where Blackness is relegated to a fleshy existence. But the people in Kishabe insist that their bodies, and the structures that care for them, have other futures too. While the hospital cannot save the lives of many that grace its doors, while the hospital sometimes even hurts its patients, people continue to come to the hospital because it performs a key function for the population in Kishabe: where so many people die unnoticed, the hospital allows Congolese the possibility to inhabit affliction, bears witness to their suffering, and teaches its patients to survive in war. Where suffering is everyday life and people have run from war more times than they can count, Congolese argue, the witnessing, the teaching to bear are themselves forms of healing. And so, the body is both the most proximate site of vulnerability and that which continues to motivate an outpouring of care, inspiring visions of presents otherwise, in which people and things can be counted on, in which life is less fragile.

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FIGURE 3.2. A store in Kishabe, emptied by retreating troops, November 2012. An untrained eye might not recognize pillage in eastern Congo. For the most part, there were no broken locks, no doors swinging open, no empty beer crates lying around. There was no evidence of struggle, nothing that would attest to thieves leaving in a hurry. Only if you had visited before would you know that an event had taken place. Windows were shuttered, doors closed, shelves cleared. Where once there had been music, neighborhood chatter, children playing between houses, the silence was deafening. Photograph by author.

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